



Clinical Practice Guideline

Oxygen Therapy

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff,

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These guidelines are intended to guide clinical decision making for clinical staff at North Metropolitan Health Service. They are not strict protocols and do not replace clinical judgement. These guidelines should never be relied on as a substitute for proper assessment of the particular circumstances of each case and the needs of each patient. If staff decide, in exercising their clinical judgment, not to follow the guideline, it is recommended that the reasons are clearly documented in the patient's health care record.

Introduction

Oxygen is a medicinal gas classified as a medicine under the Therapeutic Goods Act 1989¹. Oxygen therapy has the potential to cause harm to the patients if used inappropriately. The principles and standards for prescription, dispensing, and administration of oxygen are therefore similar to those for any other medicine.

1. Indications

The main indication for Oxygen therapy is the treatment and prevention of tissue hypoxia because of:

- a) Arterial hypoxaemia (Oxygen Saturation < 90% or PaO₂ <60 mmHg)
- b) Suspected hypoxaemia due to low pulse oximetry reading (normoxia is 94-98%)
- c) Failure of Oxygen- Haemoglobin Transport System (e.g., Anaemia, CO poisoning, shock)

2. Precautions with Oxygen Administration

No patient should be denied Oxygen therapy in an emergency or if no valid prescription is available

- Administer oxygen to maintain patient saturations at 94% - 98% and refer patient to medical staff as soon as practicable once oxygen therapy commenced for an oxygen prescription or telephone order for appropriate target saturation.
- Oxygen therapy should be prescribed cautiously to patients with severe chronic lung disease and other conditions at risk of hypercapnoeic respiratory failure (e.g., morbid obesity, neuromuscular disease, chest wall disorder, sleep apnoea), when given high percentage oxygen therapy, these patients may experience worsening hypercapnoea, reduced consciousness or respiratory arrest.
- For any patient with suspected or known hypoxia, oxygen therapy should be initiated immediately by the attending health care professional.
- The initiation of acute oxygen therapy, including indications, oxygen dose and target oxygen saturation should be documented afterwards in the patient's inpatient notes and an oxygen prescription completed as soon as practicable.³
- Oxygen therapy is administered to treat hypoxaemia, NOT breathlessness.³

3. Flow rates of Oxygen Therapy

- Oxygen therapy should be titrated to the lowest concentration that meets patient's oxygenation goals (saturation of 88–92% for patients with or at risk of hypercapnoeic respiratory failure, and 94–98% for all other patients).^{3,5} Regular close monitoring is required to maintain oxygen saturation within the prescribed target range.⁶
- For patients at risk of hypercapnoeic respiratory failure excessive oxygen therapy leading to high oxygen saturation (>24%) can worsen hypercapnoea, reduce consciousness and precipitate respiratory arrest.⁴ Exposure to high concentrations of inspired oxygen can cause absorptive atelectasis and injury to airways and lung tissue.
- Increasing oxygen requirements or decreasing saturation readings must be escalated in accordance with the Observation Response Chart (ORC).
- It is important to note that different devices deliver different FiO₂ at the same flow rates. For example, 4L of oxygen via nasal prongs will not deliver the same FiO₂ as 4L via venturi.

4. Oxygen Prescribing

- An oxygen prescription must be completed by medical staff for each patient and must include:
 - Target oxygen saturation range
 - Oxygen flow rate range
 - Delivery system / device
 - Application
 - Signature of prescribing doctor
- The prescription is to be completed on an Oxygen Prescription sticker and placed in the WA Hospital Medication Chart (WAHMC). All WAHMC signature codes apply (i.e., W – withheld)

Delivery system and standard flow rates		AM Time
☐ Nasal Prongs (NP) 0.25 - 4L/min		Sign
☐ Hudson Mask (HM) 5 - 10L/min		Flow
☐ Humidified Nasal Prongs (HNP) ≥ 5L/min		Device
☐ Other (state) _____		PM Time
Target saturations		Sign
☐ 88 - 92% ☐ ≥ 94%		Flow
☐ Other (state) _____		Device
Flow rate	Application	Sign
☐ 0.25 - 4L/min	☐ Continuous	Flow
☐ 5 - 10L/min	☐ Sleep ☐ Exercise	Device
☐ > 10L/min	☐ Wean if able	Nocte Time
Prescriber signature		Sign
Print name		Flow
If target SpO ₂ is not being met within the prescription, escalate to MO for review or activate MET as per criteria. DO NOT withhold Oxygen in an Emergency.		Device
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- Medical staff are to review the prescription daily, the rationale for ongoing oxygen therapy should be documented in the patient's medical records.
- No patients should be denied Oxygen in an emergency for want of a prescription.**
- The Nurse is to sign the medication prescription chart to verify oxygen administration during each shift indicating the prescription is acknowledged.
- The following codes indicate what device is being used for administration:
 - NP = Nasal Prongs
 - HM = Hudson mask
 - HNP = Humidified Nasal Prongs
- Flow rate is to be indicated by number, if no oxygen administered record flow rate as "0"
- In the event of "other" please use the following abbreviations:
 - NRB - Non-Rebreather mask
 - HHM – Humidified Hudson Mask
 - VM - Venturi mask
 - T - Tracheostomy
 - HTC - Humidified Tracheostomy Connector
 - NIV/CPAP - Non-Invasive ventilation

An oxygen prescription is to be completed by the specialist team (i.e., anaesthetics) if oxygen is required to be administered concurrently with prescribed therapies. i.e., opioid

infusions.

5. Exceptions to Oxygen Therapy Prescription

Patients receiving oxygen therapy who do not require a prescription include those in critical care areas such as:

- **Intensive Care Unit**
- **Emergency Department**
 - Patients who are not admitted and are being assessed for primary presentation do not require an oxygen prescription. Patient who will require ongoing oxygen therapy and are being admitted will require oxygen to be prescribed by their admitting team, or ED at the admitting teams request if unavailable to do so in person.
- **Operating theatres**
- **Post-Anaesthetic Care Unit (PACU)**
 - Patients requiring oxygen require an instruction written on the **Anaesthetic Record** under “Postoperative Instructions” by the Anaesthetists
 - Patients requiring oxygen upon discharge from the PACU are to follow the oxygen prescription written by the Anaesthetist. This is valid for 4 hours after discharge from PACU (see below - 9. Perioperative Oxygen Therapy)

6. Monitoring Oxygen Therapy

- Standard oxygen saturation target is 94–98%. Standard oxygen saturation target for patients with or at risk of hypercapnoeic respiratory failure is 88–92%.
- Where possible, obtain and document the patient’s oxygen saturation (SpO₂) using pulse oximetry before commencement of oxygen therapy.
- Patients receiving oxygen therapy must have their oxygen saturation monitored by pulse oximetry. This should be measured at least as frequently as other observations, and clearly documented on the Observation and Response Chart (MR410 871).
- NEVER discontinue oxygen therapy to obtain an oximetry SpO₂ measurement on room air in patients who clearly require oxygen therapy.³
- All patients with deteriorating oxygen saturations must be managed in accordance with the Recognising and Responding to Acute Deterioration Policy located on [SCGOPHCG Policies Hub](#).
- For patients with or at risk of hypercapnoeic respiratory failure, or with documented altered parameters on the ORC a Medical Review is required if oxygen saturation falls below the target range.
- In patients with hypercapnia, Blood gases should be repeated one-hour post commencement of oxygen therapy and as per medical direction.

7. Patient Management

- Position the patient for optimal airway and chest expansion to prevent hypoventilation and hypoxaemia/hypercarbia. Since oxygenation is reduced in the supine position, fully conscious hypoxaemic patients should ideally be allowed to maintain the most upright posture possible (or the most comfortable posture for the patient) unless there are good reasons to immobilise the patient (e.g., skeletal, or spinal trauma).³
- Pregnant women with hypoxaemia should be managed with a left lateral tilt to improve

cardiac output and oxygenation to maintain a SpO₂ of 94-98%⁵. This can preferably be achieved with manual uterine displacement or the use of a wedge.

- Assess skin integrity 4/24 hrly, including around and under oxygen tubing, Document pressure relieving interventions on the Pressure Injury Risk Management Plan Consider appropriate pressure relieving device use.
- Patients should be educated on comfort and positioning of oxygen delivery devices wherever possible.

8. Weaning and Cessation of Oxygen Therapy

- Oxygen therapy flow should be reduced as soon as the patient is clinically stable, and the oxygen saturation are maintained above the target range³ or has been at the upper end of the target range for at least 4 hours. This may require a change to the oxygen delivery device.
- Nursing staff can wean oxygen therapy if indicated on the prescription sticker.
- The potential for weaning is to be reassessed a minimum of once per day. Breathlessness is NOT an indication for oxygen therapy.³
- Oxygen saturation on room air should be monitored for at least 5 minutes after discontinuing oxygen therapy and should be rechecked at 1 hour. Oxygen therapy should be recommenced if the oxygen saturation falls below the target range.
- To cease the Oxygen Therapy prescription, it must be clearly crossed out and signed by the medical officer ceasing the therapy.

9. Perioperative Oxygen Therapy

- It is common for patients to have an increased oxygen requirement and/or reduced oxygen saturations following anaesthesia. The most common cause of this is the atelectasis that occurs as part of the effects of anaesthesia and surgery. This is usually a self-limiting condition that resolves with time. The severity of this effect is related to patient comorbidities, and the type of surgery and anaesthesia
- There are other more serious causes of reduced oxygenation that should be explored if a patient is not recovering as expected from their procedure. E.g., aspiration, pneumonia, pulmonary embolism, pulmonary oedema. It should be noted that decreased perfusion can also cause a reduction in the measured pulse oximetry
- Considering the above, it is appropriate for the anaesthesia team to prescribe oxygen for a limited duration postoperatively, in line with the expected recovery time. **The anaesthesia oxygen prescription will be valid for 4 hours from the time of discharge from PACU.** This prescription will be found on the anaesthesia chart under "Postoperative instructions".
- Oxygen saturations over 91% are generally safe so long as the cause is clear and managed appropriately. Oxygen content does not decline significantly until the saturations are below 90%.
- If a patient's condition deteriorates or their oxygen requirements increase after discharge from PACU their care should be escalated as per the "Recognising and Responding to Acute Deterioration Procedure"
- Where oxygen therapy is required for longer than 4 hours post discharge from PACU, the admitting medical team should be called for patient review to identify and manage the cause. **Oxygen therapy should be continued** until the patient is reviewed and a management plan is implemented, including oxygen prescription in the medication

chart as per this guideline.

- Patients commenced on an opioid infusion require an oxygen prescription to be completed by the prescribing anaesthetist as part of their ongoing clinical management and must be commenced by PACU nursing staff when starting opioid infusions.
- Patients at risk of hypercapnic respiratory failure are at high risk of decompensation perioperatively. These patients require significant perioperative planning and will usually be managed in HDU environments if surgery is offered at all. These patients are very unlikely to reach the wards postoperatively without being identified and having a plan in place. As such, oxygen therapy is a safe intervention on the wards postoperatively.

10. Oxygen Delivery Systems

The optimal oxygen delivery system is the one that can safely administer the required oxygen flow to achieve the target oxygen saturation

10.1 Nasal cannula (prongs)

- Nasal cannula should ONLY be used for patients who require low – medium dose (0-4L) of oxygen.
- Mouth breathers may still benefit from nasal oxygen therapy. They are generally preferred by patients over facemasks and have the advantages of improved comfort, less claustrophobia, ability to eat and speak freely, less easily dislodged, less inspiratory resistance and no risk of carbon dioxide (CO₂) rebreathing. However, this mode may not be as effective as facemasks.⁷
- Flow above 4 L/min should NOT be used as the higher flow rate tends to cause nasal dryness and discomfort.
- Nasal cannula should NOT be used in patients with nasal trauma, injury, blockage/obstruction, or epistaxis.

10.2 Hudson mask (simple mask)

- A Hudson mask (see Figure 1) is a simple facemask with open side ports that allow room air to enter and CO₂ to leave the containment space
- It can deliver oxygen therapy at a flow rate of 5-10 L/minute. Nasal prongs can be considered during interruption to continuous oxygen therapy e.g., during feeding/mouth care
- Ensure tubing is connected to oxygen flow metre and regulate oxygen flow to required rate.
- Fit mask to face ensuring that the rim of the mask fits properly over the bridge of the nose by pinching the nose-clip to provide a seal around the nose. Ill-fitting masks can lead to dry eyes. Ensure an adequate, comfortable fit.
- Assess skin integrity every 4/24, behind ears and across bridge of nose
- Check for skin irritation; ensure skin hygiene is carried out to area around mask.
- Oxygen can have a drying effect on oral mucosa: perform regular mouth care.
- Oxygen flow rate of less than 5L/min can cause rebreathing of exhaled CO₂.⁵
- Hudson mask is suitable for patients with hypoxaemia (Type I respiratory failure) but should be used with caution in Type 2 Respiratory Failure patients due to higher flow rates required.



Figure 1 Hudson mask

10.3 Venturi mask

- Venturi mask delivers oxygen by means of the venturi process and provides a predictable oxygen concentration to the patient (providing the inspiratory flow rate of the patient does not exceed that of the mask).
- The mask delivers a predetermined FiO_2 by selecting the colour-coded diluter jet and set the oxygen flow to require level. The masks are available in the following oxygen concentrations: 24%, 28%, 31%, 35%, 40%, 50% and 60%.⁷ The oxygen flow required to achieve the appropriate concentration is defined on the colour-coded diluter jets (see **Figure 2**).
- The oxygen is mixed with atmospheric air, which is already moist; therefore, there is a reduced need to consider additional humidification.

It is important that the correct flow rate is maintained.

- Increasing the flow rate will not alter the percentage of oxygen given.⁸
- A poorly fitted mask or a decrease in flow rate will reduce the accuracy of the oxygen concentration delivered.
- The accuracy of the oxygen concentration delivered is greatly reduced if the mask is not correctly placed on the patient's face, the flow is lower than recommended or if there is any obstruction to the vent.

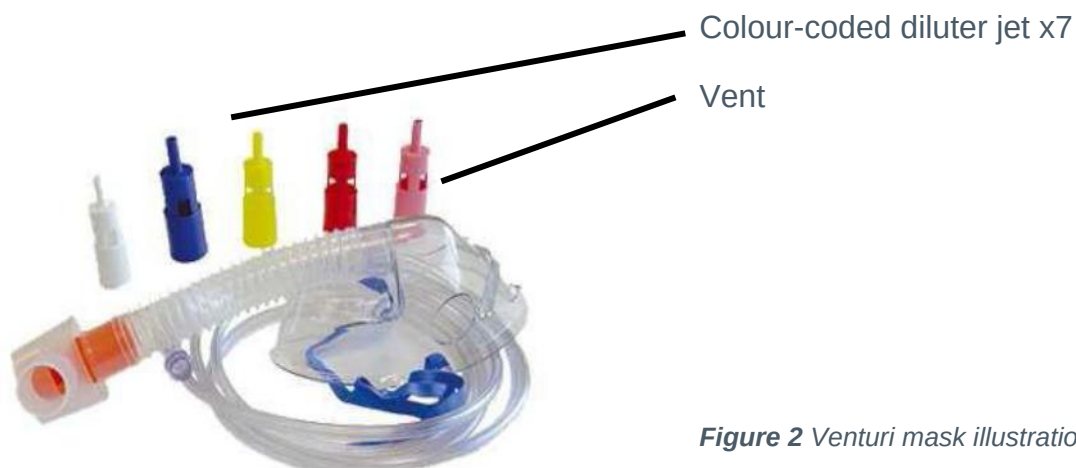


Figure 2 Venturi mask illustration

10.4 Non-rebreather mask and bag

- Is capable of delivering oxygen therapy at a flow rate of 10-15 L/minute with oxygen concentrations FiO_2 of 60-90%.⁷
- The Reservoir Bag fills with a greater concentration of oxygen available for the patient to inspire and a one-way valve preventing exhaled air from being rebreathed. If oxygen flows are interrupted, patients are at risk of asphyxiation as room air is unable to be entrained,
- Non-rebreather masks are generally only for short-term use (generally 6-8 hours) because of the high concentration of oxygen being delivered. Consider changing to a humidified high flow oxygen delivery system as soon as possible if there is ongoing high oxygen requirement.
- Core physiological observations including SpO_2 must be monitored a minimum of 4/24 and recorded on the Observation and Response Chart (ORC). In addition to monitoring of core physiological observations patients with Partial and Full Non – Rebreather Mask require:
 - Monitor and record SpO_2 at least 1/24 on the ORC.
 - Patients needing a full non-rebreather mask to maintain required SaO_2 and SpO_2 values require constant visual observation (1:1 special) and frequent review by medical staff.
 - Patients receiving oxygen therapy by partial non-rebreather mask to maintain required SaO_2 and SpO_2 values require visual observation every 15 minutes and frequent review by medical staff.
- Non-rebreather (reservoir) mask has one-way valves in the side and a reservoir bag attached (see **Figure 3**). The valves prevent air from being drawn into the mask and enables expired CO_2 to leave the mask thus preventing risk of CO_2 rebreathing. It is important that these valves remain insitu and are moving freely to achieve high levels of oxygen delivery and to prevent air entrainment.
- Non-rebreather masks are appropriate whenever severe tissue hypoxia is suspected or present.³

Figure 3 Non-rebreather mask and bag



10.5 Partial Non-Rebreather Mask

- The flow rate must be kept to a minimum of 10 L/min while using a non-rebreather mask.
- Ensure reservoir bag is filled with oxygen before fitting to patients face.
- Fit mask to face ensuring that the rim of the mask fits properly over the bridge of the nose by pinching the nose-clip to provide a seal around the nose. Ill-fitting masks can lead to dry eyes.
- Ensure a firm fit, to reduce entrainment of room air from around the edges of the mask.
- The reservoir bag should be filling and emptying with the patient's respirations. The oxygen flow rate should be sufficient to prevent the bag from completely collapsing. If the reservoir bag completely collapses, an oxygen flow meter of >15 L/min is required to enable higher flow rates to be set.
- A single valve may be removed from one of the ventilation holes on the side of the mask if patient is not continuously monitored or if oxygen interruption is anticipated. In this case the mask is now a partial non-rebreather mask and delivers a lower oxygen concentration.
- Higher concentrations of oxygen will not be maintained if the patients inspiratory flow rate is higher than the oxygen flow rate into the mask and could lead to a feeling of suffocation/air hunger and potential for worsening hypoxia.

11. Humidification

- In general ward areas a minimum flow rate of 5L/min is required for safe oxygen delivery using humidified nasal cannula.
- Heated humidification is recommended if oxygen therapy is required for greater than 24 hours at flow rates > 4L/min or as per medical orders.
- Obtain humidifier base (model: MR810) and portable stand from the Hospital Equipment Service.
- Condensation build-up in the blue corrugated tubing should be emptied into a disposable receptacle (e.g., emesis bag) ensuring the circuit is not contaminated. Discard the receptacle into general waste for disposal.
- Change whole system every 7 days or as per manufacturer recommendations, or PRN if visibly soiled.
- Attach appropriate "due change" label to the chamber and document in the Nursing Care Plan.
- Disposable humidification kit (Circuit part number RT202) containing:
 - corrugated tubing, humidifier chamber, oxygen tubing connector
 - Dual Port Adaptors (also referred to as white oxygen connector or "rabbit ears")
 - Oxygen mask / nasal prongs / direct tracheostomy attachment / tracheostomy masks are available in clinical areas

Procedure for management of equipment associated with humidification

1. Hang up the water bag on the IV pole (refer to **Figure 4: Standard set up**)

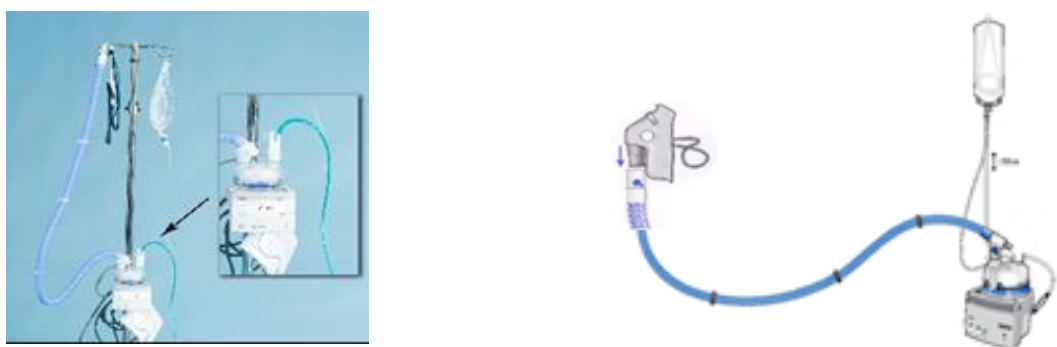
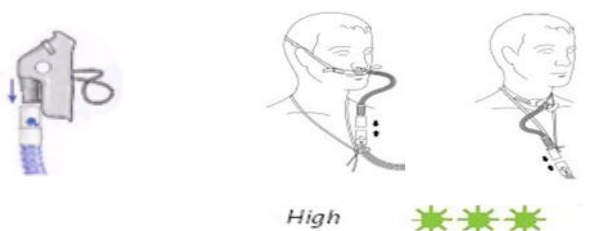


Figure 4: Standard set up

2. Remove the blue plugs from the top of the chamber and discard.
3. Uncoil the water feeder tube from the top of the chamber and discard clear plastic ring.
4. Slide the chamber on to the humidifier base.
5. Attach the water feeder tube to the water bag.
6. Attach the dual port adaptor (white oxygen connector or “rabbit ears”) to one of the chamber ports.
7. Attach one end of the oxygen tubing to the oxygen flow meter.
8. Attach the other end of the oxygen tubing to the dual port adaptor (white oxygen connector) already fitted to the chamber.
9. Plug in the heater wire protruding from the heater base into the connector of the corrugated tubing.
10. Attach the blue corrugated tubing to the other port of the chamber.
11. Attach the other end of the corrugated tubing to the desired interfacing i.e., mask, nasal prongs, direct tracheostomy connector or tracheostomy mask.
12. Set the humidifier to the desired setting. Push and hold left side of humidifier



Mask

Nasal Prongs

Tracheostomy
connector

3 lights for nasal prongs

2 lights for face mask

1 light ONLY if patient
complains it is too hot

Figure 5: Interface options

13. Turn on oxygen to required flow rate and attach the interface to the patient.
14. Ensure green light is illuminated on humidifier base.

Disposal of Equipment

1. Dispose of all consumables.
2. Clean humidifier base with detergent and water, dry thoroughly.

12. Tracheostomy mask / TRACH-VENT®

12.1 Passive humidifier with Oxygen

- Patient requiring >4 litres/minute should have heated humidification commenced via Tracheostomy mask or direct trachy attachment.
- Ensure tubing is connected to oxygen flow metre and regulate oxygen flow to required rate.
- Connect oxygen tubing to passive humidifier. Attach to tracheostomy cannula firmly, to prevent dislodgement.
- Observe patient for thickened pulmonary secretions or excessive crust formation on the tracheostomy cannula, possibly indicating inadequate humidification and need for direct humidified tracheostomy attachment.

12.2 Heated Humidification – Direct Tracheostomy Attachment

- Used to deliver oxygen and humidification to the patient via the tracheostomy. In general ward areas, a flow rate minimum of 5 litres / minute is required for safe oxygen delivery using heated humidified direct tracheostomy attachment.
- For patients with oxygen requirement below 5 litre / minute an alternative oxygen delivery device should be used (i.e., Passive humidifier).
- Humidified oxygen delivered via direct trachy attachment must be heated using a humidification unit available from the hospital equipment store.
- Ensure tubing is connected to oxygen flow metre and regulate oxygen flow to required rate.
- Attach tracheostomy interface firmly to tracheostomy cannula to prevent dislodgement.

Related Documents

- [Health Practitioner Regulation National Law \(WA\) Act 2010](#)
- [Australian Health Practitioner Regulation Agency \(AHPRA\) Code of Ethics for Nurses August 2008](#)
- [AHPRA Code of Professional Conduct for Nurses August 2008](#)
- [Poisons Act 1964, Poisons Regulations 1965 and Poisons Amendment Regulations 2010](#)
- [Occupational Safety and Health Act 1984](#)
- [NMHS Clinical Documentation Policy](#)
- [NMHS Infection Prevention and Control Policy](#)
- [Recognising and Responding to Acute Deterioration Procedure](#)

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form